

OPENING (2 min) — Say this first

"Hi [Name], thanks for making time. We're researching how families and care professionals manage day-to-day care for seniors at home. We're not selling anything — we're here to learn from your real experience. No right or wrong answers. Is it okay if I take notes / record? We have [30/45/60] minutes. Does that work?"

WARM-UP (5 min) — Context & Background

- "Tell me about your role — what does a typical week look like for you?"
- "How long have you been caring for / working with seniors at home?"
- "How many people are you responsible for right now?"
- (FC only) "Are you the primary contact, or are other family members involved?"
- (HCN/CC) "What tools does your agency currently use to track patient status?"

BLOCK 1: FALLS

- "Walk me through the LAST TIME someone you care for had a fall. What happened from the very beginning?"
- "What did you do first? Then what? Who else was involved?"
- "How long did the whole response take?"
- "What was the hardest part of that situation?"
- "What info did you wish you had but didn't?"
- "Was there a moment you weren't sure whether to call 911 or wait?"
- "Have you ever gotten a false alarm? What happened?"
- "What would have made that situation easier or safer?"

BLOCK 2: MEDICATIONS

- "Tell me about the last time there was a problem with a medication — missed dose, wrong dose, confusion."
- "How did you find out? How long after it happened?"
- "What did you do? Who else was looped in?"
- "What's the most frustrating part of keeping track of medications?"
- "What do you use today to manage this? How does it break down?"
- "Have you been unsure whether a dose was taken or not? What did you do?"
- "If this worked perfectly, what would that look like day to day?"

BLOCK 3: COMMUNICATION & ESCALATION

- "When something concerning happens, how does info get to you? Walk me through the chain."
- "What's the weakest link — where does info get lost or delayed?"
- "Have you ever second-guessed a decision to escalate or not? Tell me about that."
- "How do you decide: emergency vs. watch-and-wait?"
- (HCN/MD) "When you get a device alert, how much do you trust it?"

BLOCK 4: CURRENT TOOLS & WORKAROUNDS

- "What tools/systems do you use to monitor or coordinate care?"
- "Walk me through how you used [tool] last week."
- "What does it do well? Where does it fall short?"
- "Have you tried anything new in the last year? What happened?"
- "What would you give up to have a system that actually worked?"

BLOCK 5: WILLINGNESS TO PRIORITIZE (Do NOT pitch — past behavior only)

- "How much time per week do you spend on coordination and communication around care?"
- "Does your agency/family currently pay for any monitoring technology? What?"
- "Have you ever evaluated a care tool and decided NOT to buy? What made you say no?"
- "If a tool solved [biggest pain], what would a fair price look like?"

When You Hear...	Probe With...
Vague answer	"Can you give me a specific example?"
Strong emotion	"Tell me more about that feeling. What was at stake?"
Surface answer	"Why?" (ask gently, up to 3x)
Story stops short	"What happened next?"
Mentions someone	"Who is that person? How did they fit in?"
Mentions workaround	"How long have you done it that way?"

THE MOM TEST — ACTIVE REMINDERS

Ask about THEIR LIFE, not your idea
 Ask about THE PAST — never "Would you use X?"
 Talk less, listen more — target 80/20 split
 NEVER PITCH during the interview
 Compliments are noise — "Sounds useful!" = nothing
 Strong emotions = REAL SIGNAL — dig in immediately

WRAP-UP (3 min)

"Is there anything I didn't ask that you think is important?" | "Who else should I talk to?" | "Can I follow up with you?" | Thank them. Promise to share findings.

INTERVIEW NOTES | Home-Care-AI | Fill in during or immediately after session

Participant ID:	_____	Persona:	FC / HCN / CC / MD / SR
Date:	_____	Duration:	_____ min
Interviewer(s):	_____	Session #:	_____ of _____

KEY JOBS — What are they fundamentally trying to accomplish?

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CURRENT SOLUTION — What do they use today? (tools, workarounds, people)

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BIGGEST PAIN — Use their EXACT WORDS (verbatim quotes only)

Rule: Key insights must use exact customer words. No paraphrasing. Synthesis goes below the quote.

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DESIRED OUTCOME — What does 'good' look like in their own words? (verbatim)

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ESCALATION BEHAVIOUR — How/when do they decide to escalate vs. wait?

Feeds HITL threshold calibration and Agentic Control Matrix (L1/L2/L3)

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EMOTIONAL INTENSITY — Moments of strong frustration, fear, relief, resignation

Strong emotions = real signal. These become the priority pain signals.

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WILLINGNESS TO PRIORITIZE — Time/money spent? Paid for anything similar?

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TOOL MENTIONS — Specific products, apps, or systems named

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SURPRISE FINDING — Something unexpected or that challenges an assumption

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REFERRAL — Who else should we talk to?

FOLLOW-UP ACTIONS

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NEXT STEP: After 3+ sessions with the same persona, run `summarize-interview` in your Discovery Claude Project

Upload these raw notes. Output: JTBD map, verbatim quote bank, pain signal ranking → feeds competitive-gap-analysis